

How I Want You to Present — SOAP

Trauma, Acute Care & General Surgery · Christopher Gonzales, MD

Same skeleton, every patient, every day. Present in **SOAP** order and in **this** order within it. Tell the story, then give me one problem-based plan.

S — SUBJECTIVE

- 1. One-liner** — age/sex, POD#/HD# + procedure, the core problem, where they're trending. One breath.
- 2. Quick recap — major events** — the admission story in ~2 sentences. Not a day-by-day recital.
- 3. Yesterday's plan → what happened** — what we said we'd do, whether it happened, + brief, relevant overnight events.
- 4. Subjective today** — how the patient actually is this AM: pain, N/V, flatus/BM, appetite, complaints.

O — OBJECTIVE

- 5. Vitals** — pertinent (Tmax, ranges, trend); call out fever / tachy / hypotension / low UOP.
- 6. Exam** — focused: wound, abdomen, drains, lines, the system in question.
- 7. Pertinent labs** — only what matters, **with the trend**. Not a wall of numbers.
- 8. Pertinent imaging** — the study that changes management + its read.

A / P — ASSESSMENT & PLAN

- 9. ONE problem-based list** — a single numbered problem list, most important first, each with its plan. Not by organ system — one integrated list. **Every plan is concrete** (drug · dose · route · frequency), and **every line, tube & drain** gets a plan — IV/fluids, Foley, drains: keep, change, or pull.

Match length to acuity: ~60–90 sec on a routine patient, more on the sick one. Sickest first.

WORKED EXAMPLE (ILLUSTRATIVE)

One-liner: "69M, POD#6 from ex-lap washout and liver necrosectomy for infected necrosis — septic, now with a worsening AKI, trending the wrong way on renal function."

Recap: "Cholecystectomy last month, hepatic bleed needing embolization, then this washout; multiple drains and a right chest tube in."

Yesterday → overnight: "Plan was to follow GI's scope recs and hold heparin for his GI bleed — heparin was stopped; overnight a couple more dark stools but he stayed hemodynamically stable."

Subjective: "Tired but comfortable, no new pain, tolerating clears."

Vitals: "Afebrile now, Tmax 38.1, HR 90s, MAPs >65, still making urine but down from yesterday."

Exam: "Abdomen soft, 3 drains serosanguineous, chest tube to waterseal, no new erythema."

Pertinent labs: "Cr 5.5, up from 3.9; BUN 79 — worsening. Hgb 8.2, single value so I'm rechecking. WBC 10.7, down."

Imaging: "AM CXR — minimal pleural fluid, chest tube can come out."

Assessment / plan (one list):

#1 Worsening AKI — renal-dose the antibiotics, hold nephrotoxins, LR 500 mL bolus, renal consult if no better.

#2 GI bleed — heparin held, pantoprazole 40 mg IV BID, trend Hgb q8h, transfuse for Hgb <7.

#3 Intra-abd infection — 3 drains to bulb suction, record outputs, narrow abx per cultures.

#4 Lines / tubes / drains — R chest tube to waterseal, pull if AM CXR stable; central line still needed (abx/labs); IV maintenance 75/hr, wean as PO improves; Foley to track UOP.

#5 Nutrition / dispo — advance to clears, PT, remains ICU.

Notice: every plan is concrete (drug/dose/route/freq), and every line/tube/drain is accounted for. Labs are only the movers, each with its prior; the plan is one ranked problem list, sickest first — not Neuro/CV/Pulm blocks.